

Population aging and intergenerational relationships

Social aspects of Population aging

Social implications of population aging

- Definition of care
- Types of care
- Measuring care provision
- Valuing care
- Care needs
- Care Regimes
- Providing care in ageing societies



“If we look at history through the lens of care and relationships, there are no breaks. As a society, we haven't changed that much”

Raquel Licerias (PhD in Prehistory), Universidad Complutense de Madrid



What is care?



***“(...) everything that we do to
maintain, continue, and repair
our 'world' so that we can live
in it as well as possible”***

Fisher and Tronto (1990)



Then again what is care?



What is care?

- **Direct physical care:** e.g. bathing, dressing, feeding.
- **Household work:** e.g. cooking, cleaning, shopping.
- **Medical care:** e.g. administering medications, managing doctor appointments.
- **Emotional and relational support:** e.g. companionship, reassurance.
- **Instrumental help:** e.g. helping with phone use or internet, managing finances.
- **Mental load of care:** e.g. planning care, managing care needs, anticipating them.



What is care?

- The definition of care would depend on the objective of the research study
- Looking to the capability of the care recipient to be dependent?
- Regarding the health / labor activity / well-being of caregivers?



Names of care

- Support
- Help

Do they define the same?

- But in general we are focusing on its provision, so maybe refer to **Carers / Caregivers** would be a better choice.



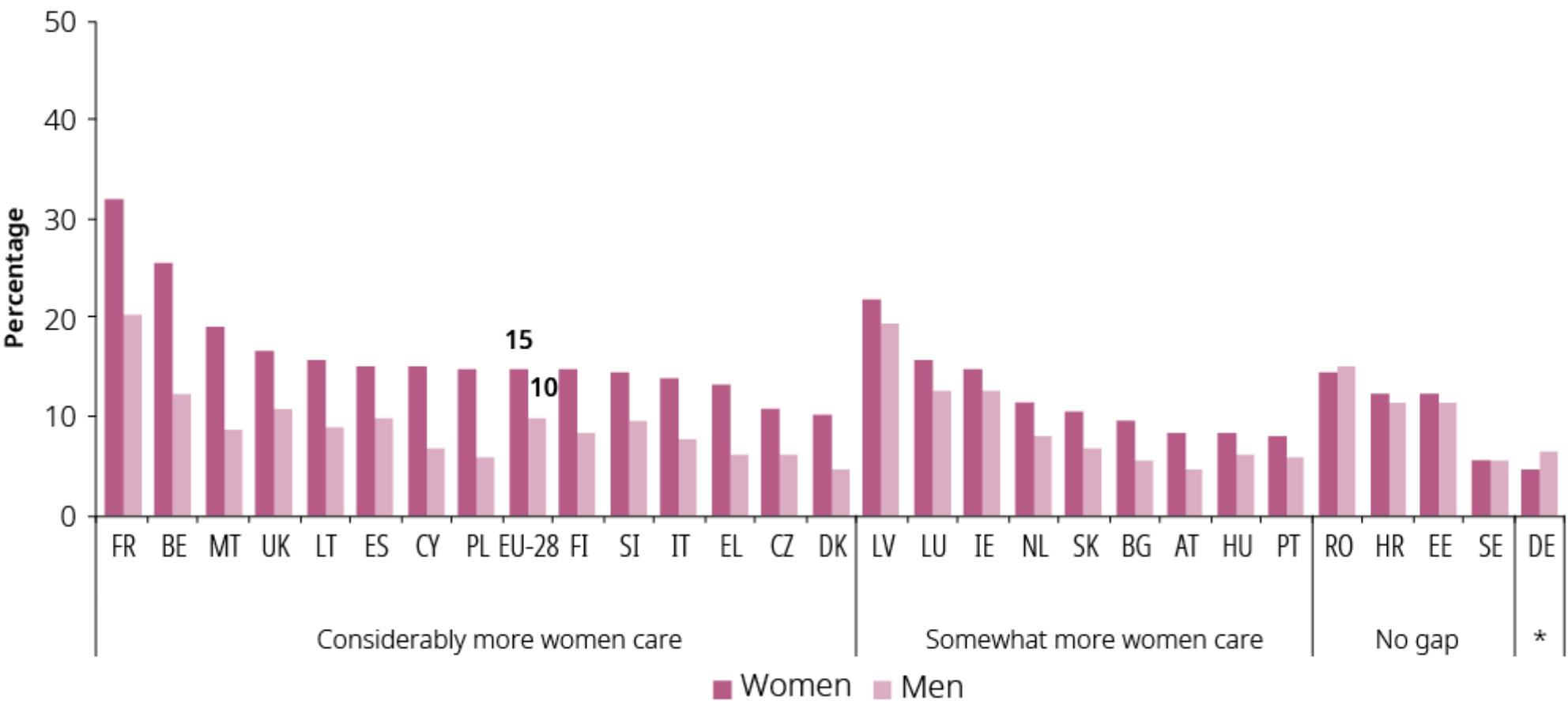
Types of care

- Formal / Informal
- Paid / Unpaid
- Adult / Children
- Women's role
- Frequency and Intensity of care is crucial

The combination of these different types of care in each country conforms what has been called as **care regimes**

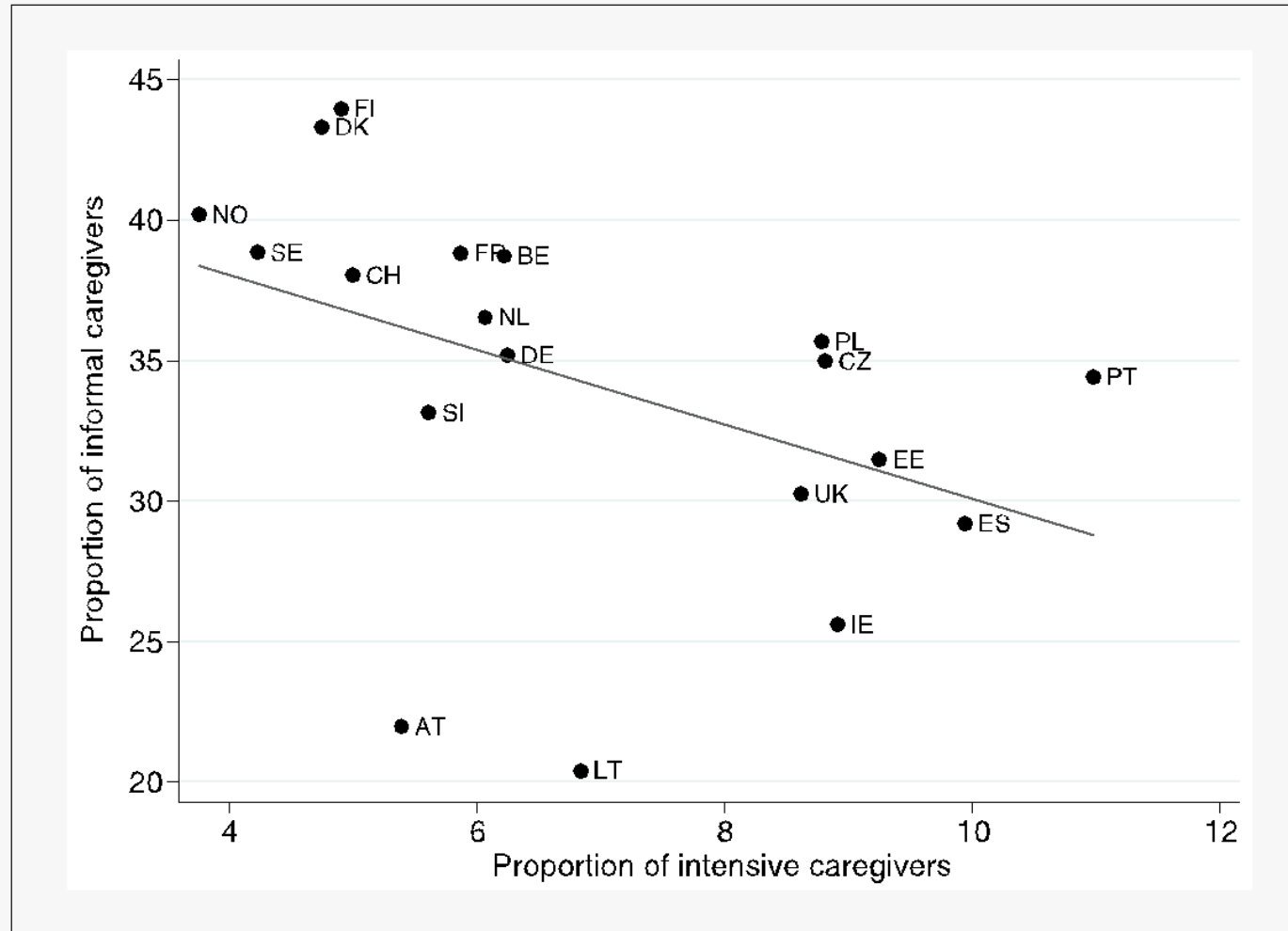


Proportion of adult caregivers 18+ by gender



Source: European Institute for Gender equality

Proportion of informal caregivers over care intensity

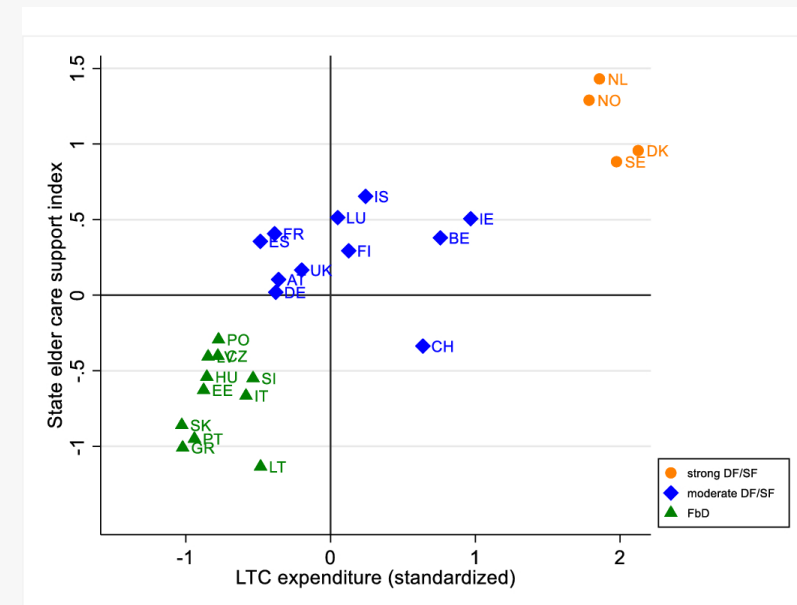
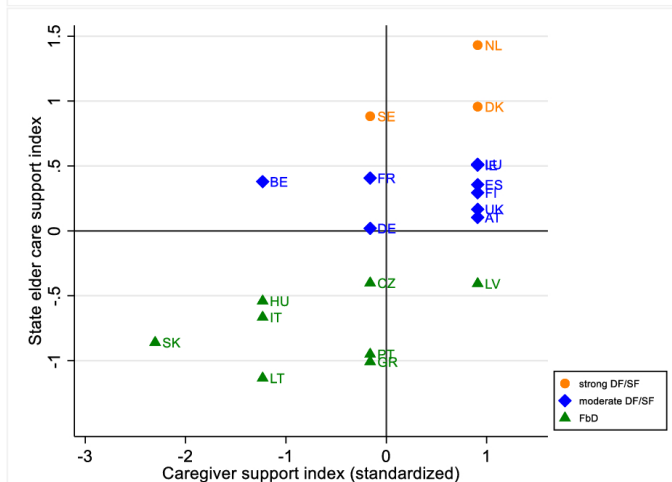
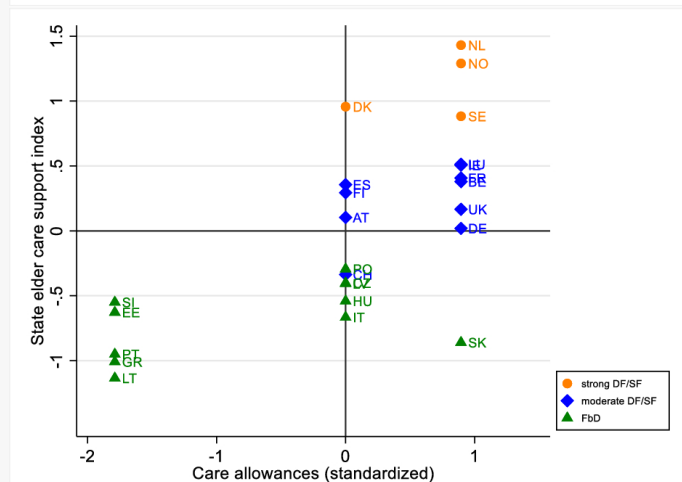
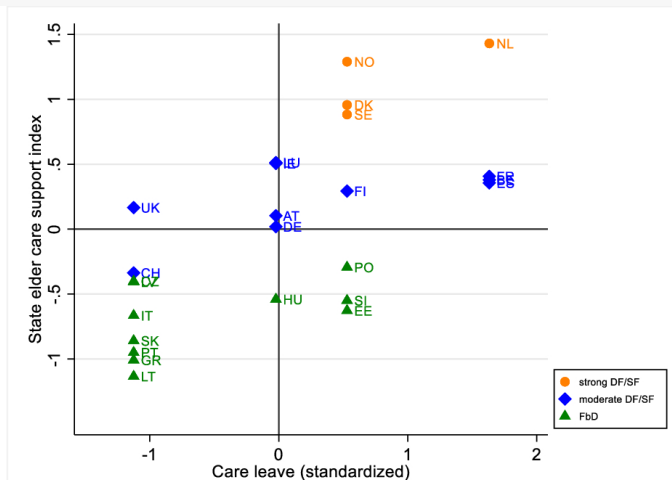
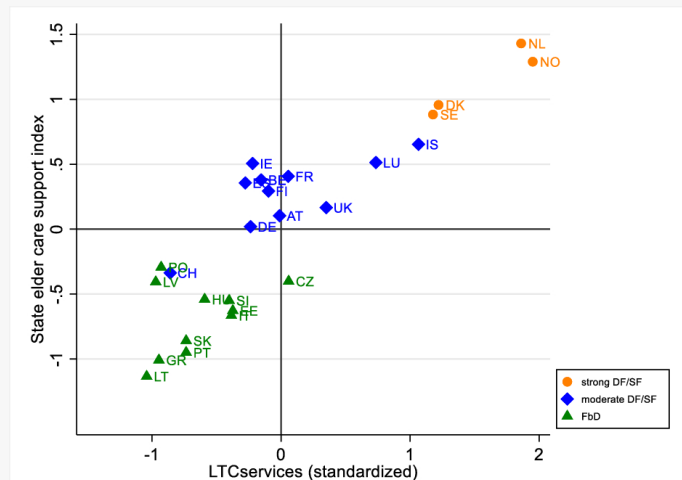


Verbakerl, 2017. How to understand informal caregiving patterns in Europe? The role of formal long-term care provisions and family care norms. *Scandinavian Journal of Public Health*, 2018; 46: 436–447

Care regimes

- Esping-Andersen (1990; 1999) Welfare states: Liberal / Conservative / Social democratic - not much information about care
- Leitner (2003) introduces the concept of Familialism / Defamilialism regarding care
- Saraceno & Keck (2010): Familialism by default / Supported familialism / Defamilialization
- Pfau-Effinger (2005): Formal and informal institutions (cultural norms) and the gender culture
- Based on all these concepts, van Damme et al. (2025) proposed a new care regime typology, taking a special look at the gender roles.





van Damme, M., Spijker, J., & Pavlopoulos, D. (2025). A care regime typology of elder, long-term care institutions. European Journal of Ageing, 22(1), 20. <https://doi.org/10.1007/s10433-025-00854-0>



Care regimes

Regime Type	Definition / Main Logic	State Role	Family Role	Typical Country Examples
Defamilialism	State or market substitutes for the family in caregiving. Care is decommodified and publicly/market provided.	Strong provider of services (“care in kind”)	Low – individuals are not dependent on family	Sweden, Denmark, Finland
Supported Familialism	State supports family caregiving through cash benefits, leave, and tax relief, but family remains main provider.	Moderate – enabler of family caregiving	High – family (often women) expected to care	Germany, Austria, France, Belgium; Netherlands, Switzerland
Familialism-by-Default	Weak or absent public support; care falls to the family by necessity.	Minimal – residual or fragmented	Very high – family obliged to provide care	Italy, Spain, Greece, Portugal, Poland



Measuring care (unpaid and informal)

Many times defined as invisible, its measurement is crucial to value the contribution of women to productive activities

- Time diary method: Participants record all their activities in a time diary over a defined period, usually a single day (24 hours).
- Time use surveys: TUS or HETUS



Measuring care (unpaid and informal)

- Recall method: Uses stylized questions about the time devoted to a list of tasks in a reference period (a day, a week, a month, a year).
- Example: SHARE



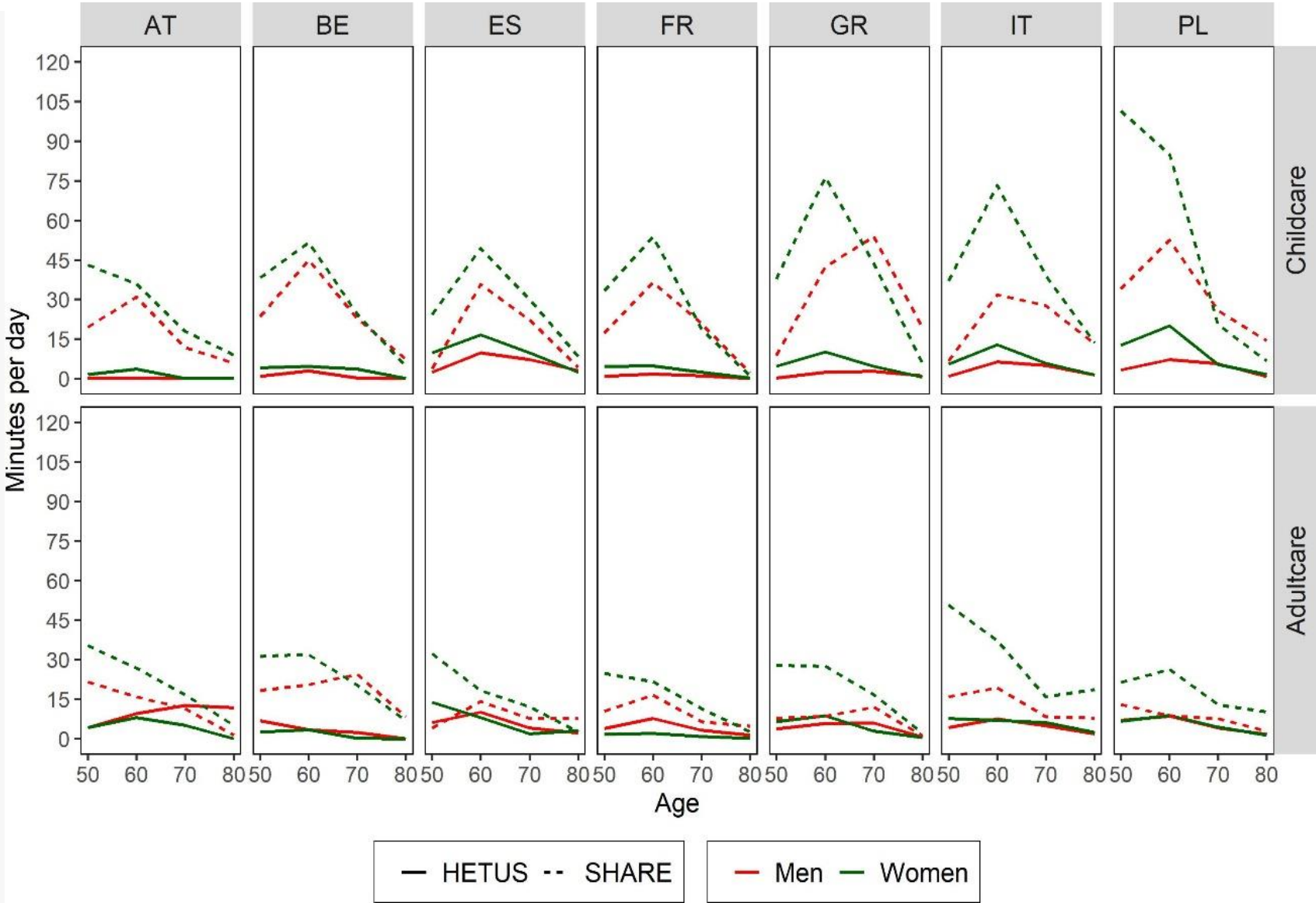
	PROS	CONS
Time diary	• Very specific and accurate • More reliable	• More expensive to apply • How they capture sporadic activities • Simultaneous activities
Recall method	• More detailed in the type of activities • Broader periods of reference • Capture more sporadic activities	• Less exact in time measurement • More subjective



Measurement of
care done to
non-co-resident
members:

HETUS vs.
SHARE

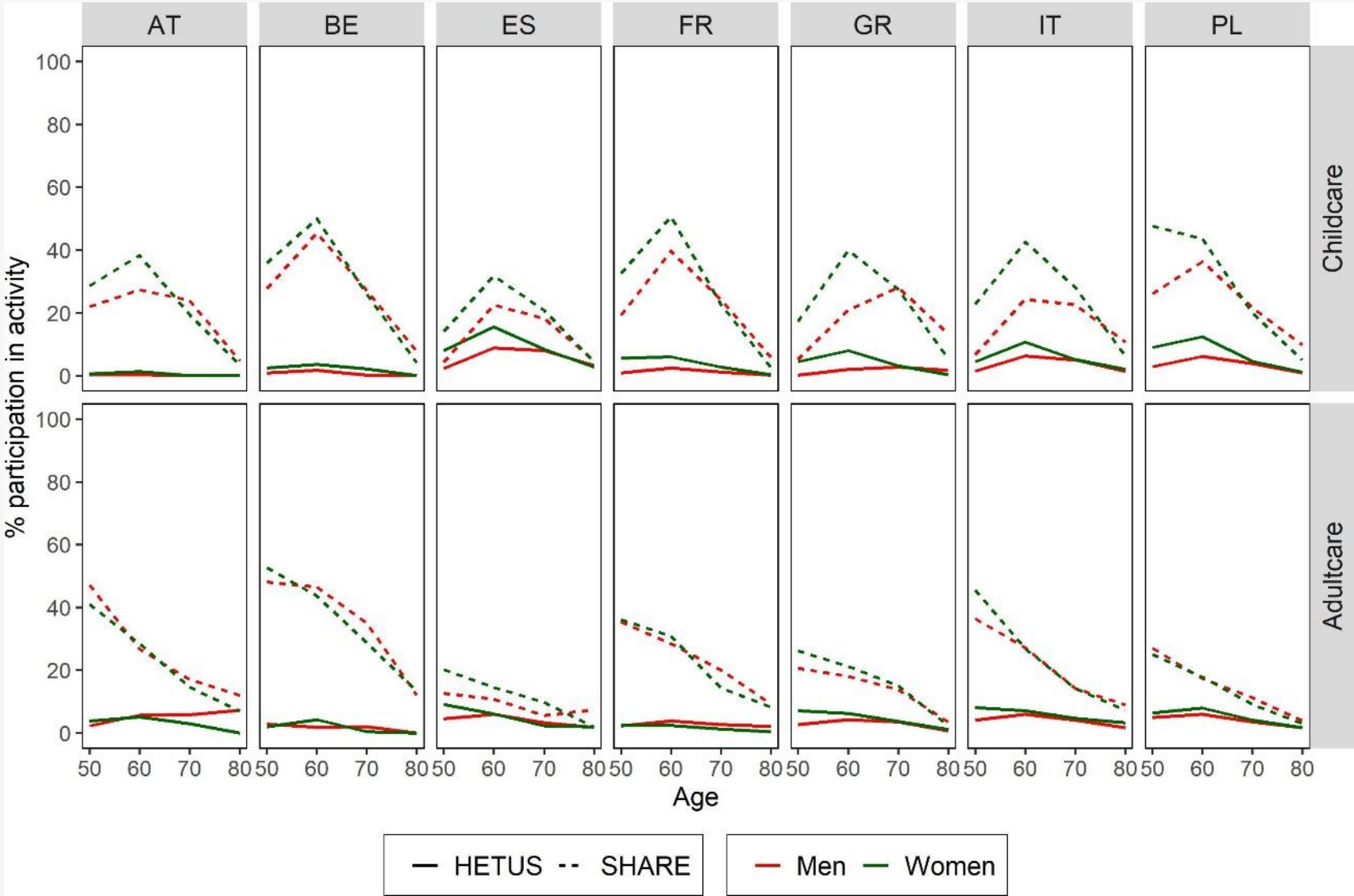
Average time provided



Measurement of
care done to
non-co-resident
members:

HETUS vs.
SHARE

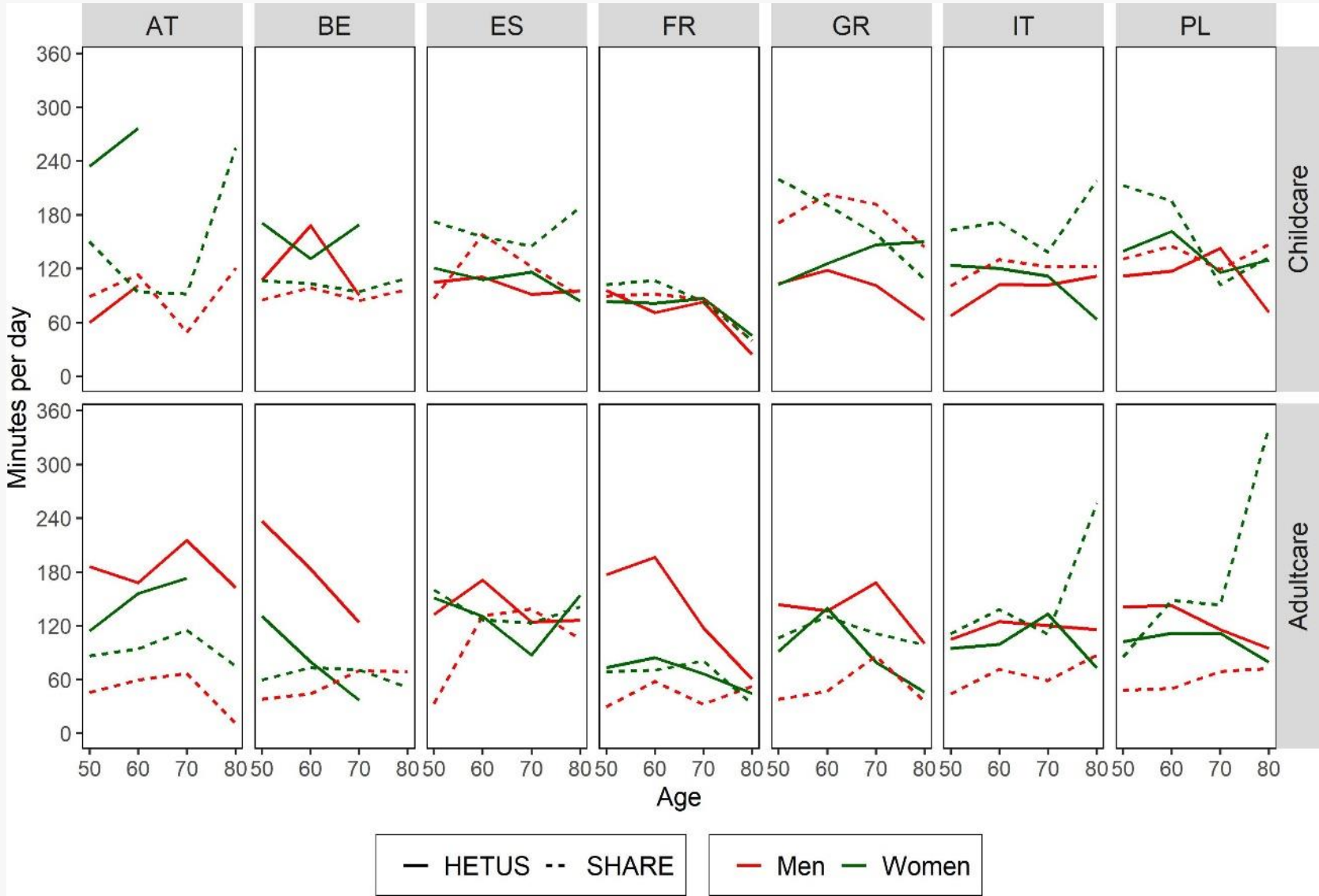
Proportion of caregivers



Measurement of
care done to
non-co-resident
members:

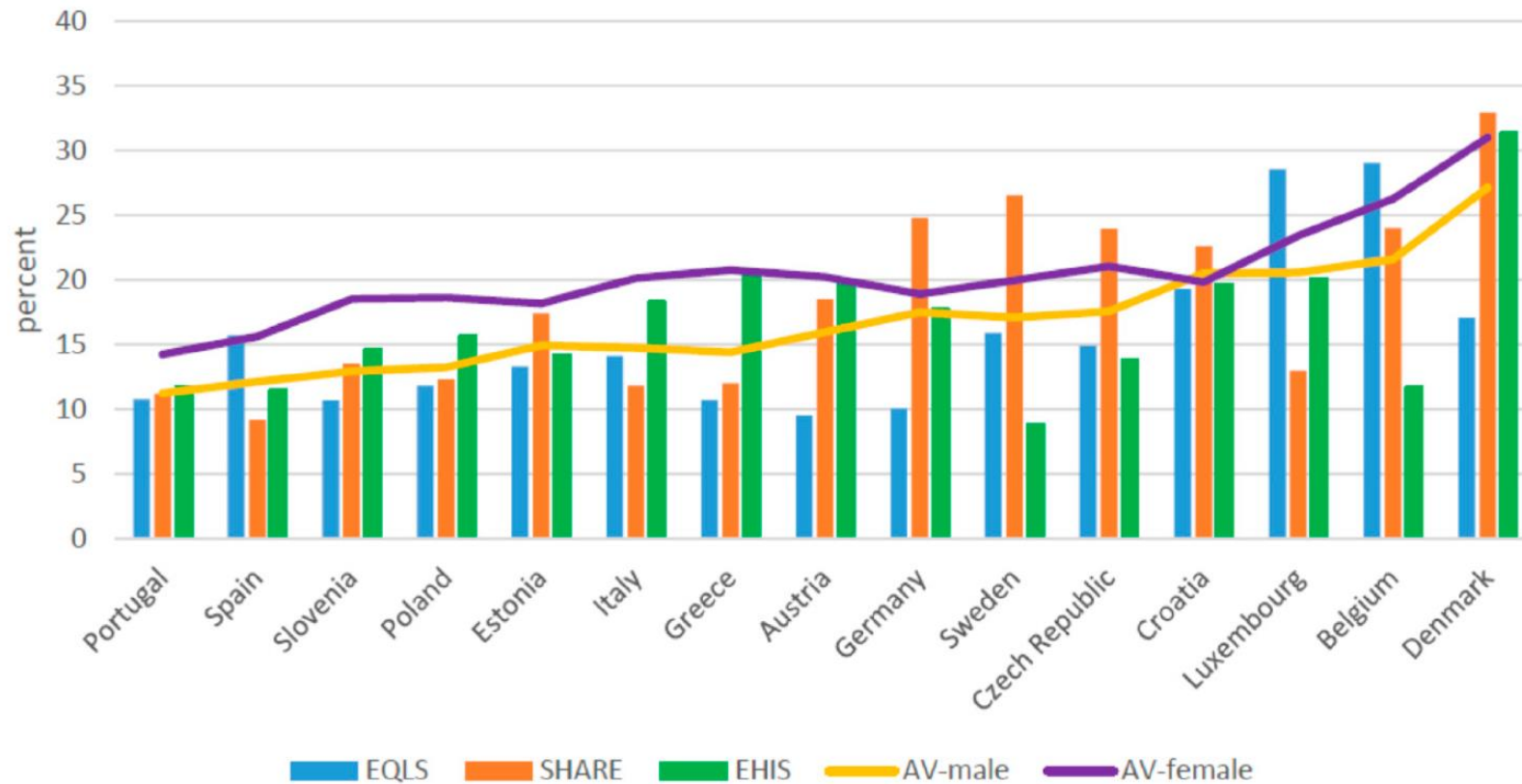
HETUS vs.
SHARE

Average time provided by each caregiver



Differences also in the recall method

Informal MALE Caregivers 50+ females in EQLS, SHARE and EHIS

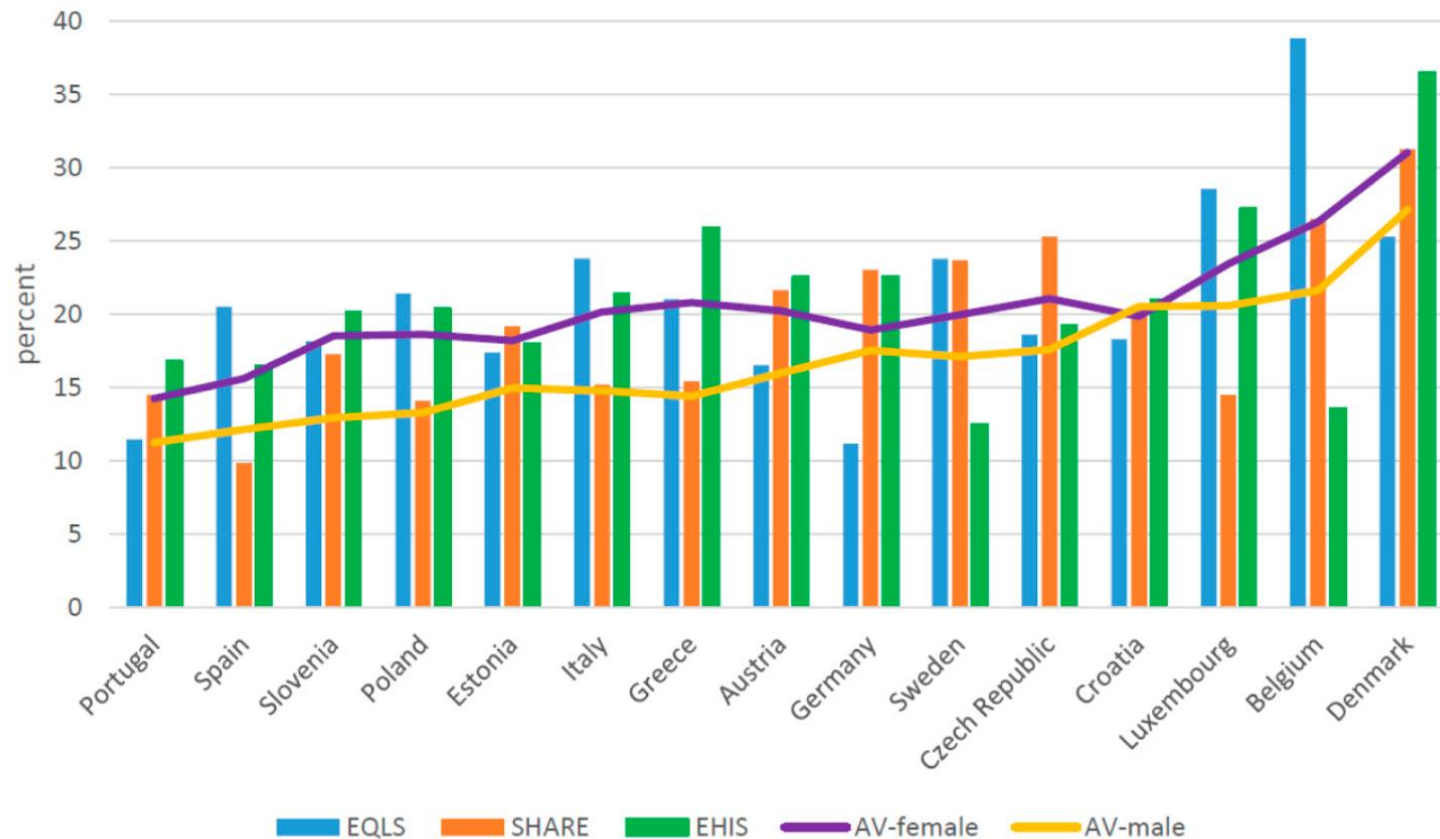


EQLS = European Quality of Life Survey
SHARE = Survey of Health, Ageing and Retirement in Europe
EHIS = European Health Interview Survey
AV=Average



Differences also in the recall method

Informal FEMALE Caregivers 50+ females in EQLS, SHARE and EHIS



EQLS = European Quality of Life Survey
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EHIS = European Health Interview Survey
AV=Average



Valuing care

- **Replacement cost method (Most used one):** Cost of paid services.
- **Opportunity cost method:** Measures the income the caregiver loses, therefore understanding the loss of market production.
- **Willingness to pay:** Needs and specific question.



From care providers to those in need of care

Care Needs

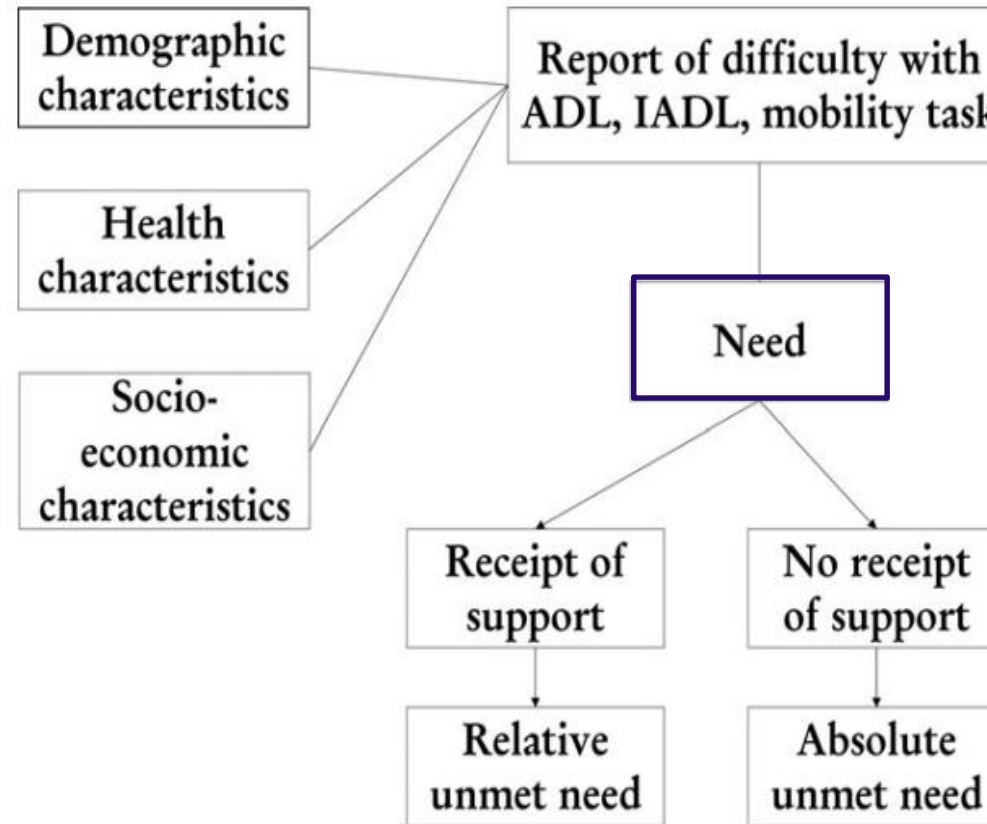


Figure 1. Conceptualising unmet need for social care.

Notes: ADL: activity of daily living. IADL: instrumental activity of daily living.

Source: Vlachantoni, 2019



Measuring Care Needs

- **Mobility:** Walking 100 m; Sitting for about two hours; Getting up from a chair after sitting for long periods; Climbing several flights of stairs without resting; Climbing one flight of stairs without resting; Stopping, kneeling, or crouching; Reaching or extending your arms above shoulder level; Pulling or pushing large objects like a living room chair; Lifting or carrying weights over 10 pounds/5 kilos, like a heavy bag of groceries; and Picking up a small coin from a table.
- **ADL & IADL:** Walking across a room; Bathing or showering; Eating, such as cutting up your food; Getting in or out of bed; Using the toilet, including getting up or down; Using a map to figure out how to get around in a strange place; Preparing a hot meal; Shopping for groceries; Making telephone calls; Taking medications; Doing work around the house or garden; Managing money, such as paying bills and keeping track of expenses; Leaving the house independently and accessing transportation services; and Doing personal laundry.



Unmet Care Needs

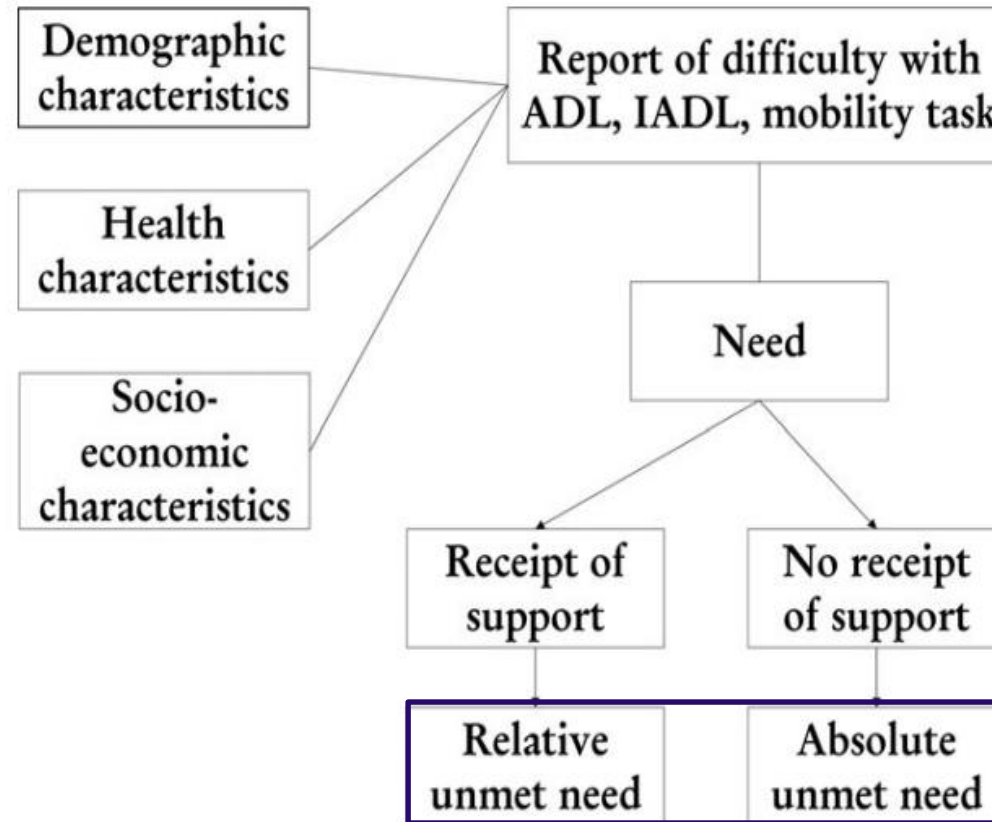


Figure 1. Conceptualising unmet need for social care.

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From care providers to those in need of care

Unmet Care Needs

- Unmet needs refer to nature and quantity of care that is needed but not being received (Vlachantoni et al 2011).
- Two approaches and two strong assumptions:
 - Not receiving support implies an unmet need.
 - Perceptions about received support are accurate to measure unmet needs

Source: Vlachantoni, 2019



Relative unmet care needs and care quality

- Challenging to measure it.
- Most of the evidence comes from a qualitative perspective.
- A research gap to contribute.
- The meaning of the quality can change from one scenario to another.



Presentations Time!!!



Debate on the availability of caregivers for older individuals



Options for Long-term care

1. Over recent decades, many European countries have embraced **deinstitutionalization**, replacing traditional care with community-based services (Ilinca et al., 2015).

2. Respect for human rights:

- Community living supports autonomy and access to basic rights, which are often denied in institutional settings.
- Non-institutional services empower individuals with disabilities to maintain independence and to participate in family, community, and economic life.

3. Sustainability and efficiency:

- Evidence shows better outcomes for users and families, at lower or comparable costs;
- Community-based care also aligns with public preferences: Europeans across countries overwhelmingly prefer to age in place.



Availability of caregivers

As the care needs of the growing elderly population are met by family and friends, governments worldwide are recognizing the critical importance of supporting these informal caregivers and safeguarding their health and well-being.



Availability of caregivers

Demographic Supply-Side Factors

- Aging Population
- Birth Rates

Socioeconomic Factors

- Gender Roles
- Labor Force Participation
- Financial Incentives/Support
- Education and Training

Personal and Family Dynamics

- Family Structure
- Caregiver Burden
- Health and Well-being of Potential Caregivers
- Cultural Norms



Impact on health of caregivers

Women disproportionately provide informal care for dependents.

While kinship ties offer crucial emotional and instrumental support—acting as intrafamilial safety nets that facilitate resource exchange and protect against adverse outcomes (Carr & Utz, 2020; Freedman et al., 2024; Martinez-Marcos & De la Cuesta-Benjumea, 2024))—this role comes at a significant cost.

Impacts on health and well-being:

- Worsened general health status
- Higher prevalence of chronic conditions such as angina, arthritis, and hypertension
- Lower perceived quality of life
- Increased psychological distress, including anxiety and depression
- Many caregivers are older and already managing chronic health problems
- Prolonged caregiving associated with higher morbidity and mortality





HOW TO COMBAT CAREGIVER BURNOUT

WHY IT MATTERS

Caregiving for a loved one is rewarding, but can also result in prolonged and overwhelming stress for the caregiver. Caregiver burnout refers to a state of emotional, mental, and physical exhaustion that can be very damaging to the family. People who experience caregiver stress can be vulnerable to changes in their own health. Here are a few tips on avoiding burnout:

FOCUS ON WHAT YOU ARE ABLE TO PROVIDE.

Don't stress out over things you can't control. Prioritize, make lists and establish a daily routine. Break down larger tasks into more manageable steps. Remember: all your efforts, big and small, matter!



LEARN WAYS TO BETTER HELP YOUR LOVED ONE.

Look for caregiving resources in your community, including classes at the hospital or caregiving services such as transportation, meal delivery, or housekeeping.

GET CONNECTED.

Talk to a supportive family member or friend. Some communities offer respite services. Find support groups where you can share stories, get caregiving tips, and get support from others who face the same challenges.



SET PERSONAL HEALTH GOALS.

Take care of your own health and set goals to establish a good routine. Don't skimp on exercise or sleep. Make time to maintain your friendships, relationships, and hobbies so that you can stay healthy and happy.

ACCEPT HELP

Make a list of ways that others can help you. If possible, spread the responsibility with tasks that other family members and friends can help with. Consider how in-home senior care might help your loved one.



Neighborly Home Care was founded to provide top-quality home health care services in Philadelphia, with an emphasis on providing highly-trained caregivers who will give personalized care based on the needs of your elderly loved ones.

WWW.NEIGHBORLYHOMECARE.COM

YOU DESERVE A BREAK!



55% of caregivers feel overwhelmed by the amount of care needed for a family member.



Caregivers spend an average of 18 hours per week providing care to a family member.



1 in 5 caregivers report physical strain due to caregiving duties.

5 TIPS TO MANAGE CAREGIVER ANXIETY & DEPRESSION



CAREGIVER ANXIETY

Caregiver stress and anxiety can be caused by managing too many responsibilities, loss of control, fear of a loved one's well-being, uncertainty about the future or worrying about healthcare finances.



CAREGIVER DEPRESSION

Caregivers often spend more time taking care of others needs over their own. Neglecting your needs to care for loved ones over a prolonged period, combined with strained or overwhelming work environments, can have serious mental health consequences and can result in depression.



TIPS TO MANAGE CAREGIVER BURNOUT

PRACTICE ACCEPTANCE

When faced with the unfairness of a loved one's illness, practice acceptance and try to avoid the emotional trap of feeling sorry for yourself or searching for someone to blame.

ASK FOR HELP

Don't be afraid to ask others around for help or support.

FIND A BALANCE

Make sure caregiving doesn't take over your life. Try to incorporate self-care practices into your routine if you begin to feel overwhelmed.

CELEBRATE YOURSELF

Find ways to acknowledge and remind yourself of all the help and effort that you are putting forth. It is important to celebrate the small victories and acknowledge your support as a caregiver.

FOCUS ON THINGS YOU CAN CONTROL

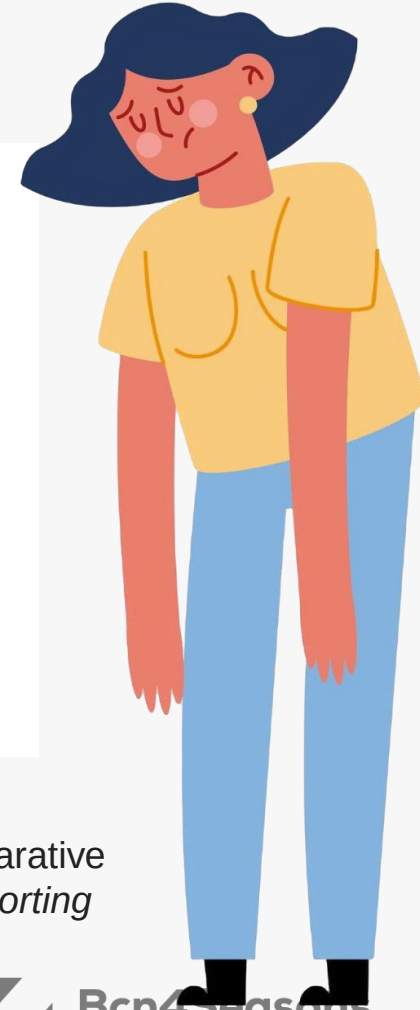
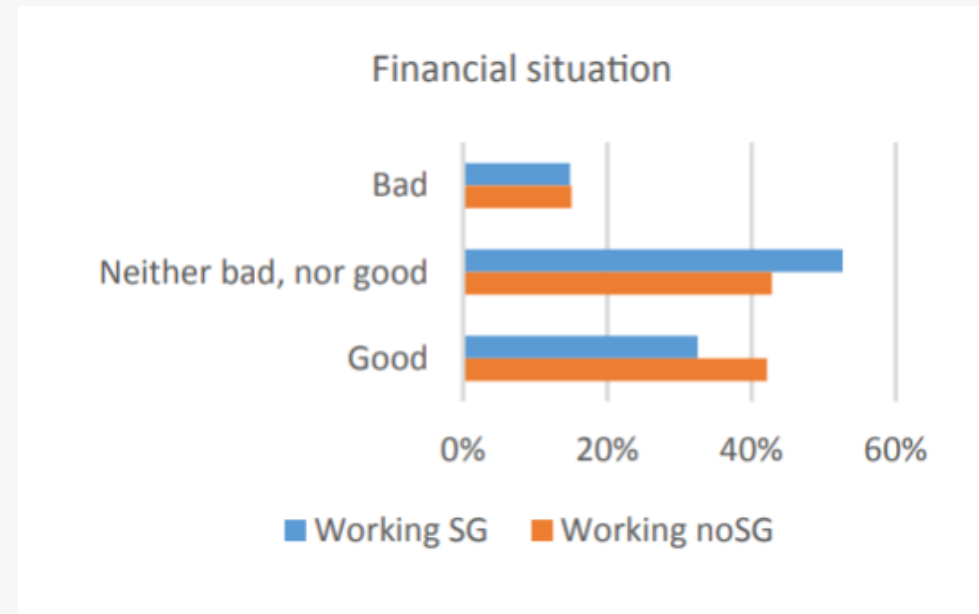
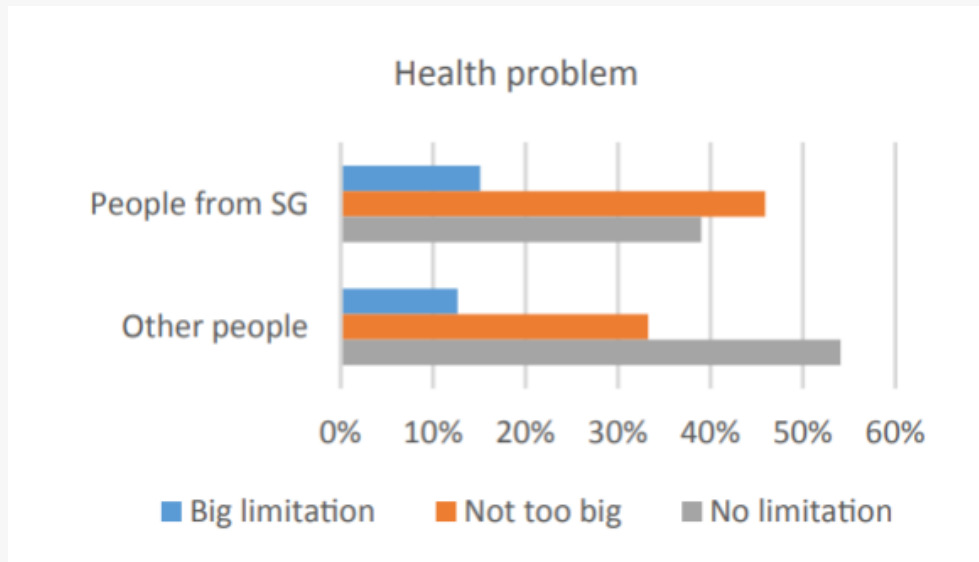
Rather than stressing over things you can't control, like wanting more hours in the day, focus on how you chose to react to problems when they present themselves.

Check out these helpful resources to learn more about [caregivers and anxiety and depression](https://www.adaa.org). For more information visit:

[WWW.ADAA.ORG](https://www.adaa.org)

ADAA

Consequences on the Sandwich generation of women



Kwiatkowska-Ciotucha, D., & Załuska, U. (2022). Sandwich Generation in the Workplace–International Comparative Research. In *Working Women in the Sandwich Generation: Theories, Tools and Recommendations for Supporting Women's Working Lives* (pp. 109-127). Emerald Publishing Limited.



Alternatives fro caregivers offer

1. Facing the growing challenge of caregiver scarcity in the future, there's significant uncertainty regarding solutions, especially at governmental levels. However, literature already points to alternative practices that can mitigate the effects of this shortage, such as cohousing and care robots.



“Social Care isn’t Rocket Science, is much more complex than that”

(Beresford, 2005)



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